

Constipation

WARWIKI

Patient Instructions · Why it matters for pelvic health, and how to fix it

Constipation means stools that are **infrequent, hard, or hard to pass**. It is very common — and it matters for pelvic health because the **straining** it causes worsens prolapse, bladder leakage, and pelvic pain. The good news is that most constipation improves with simple, proven changes.

About This Condition

Healthy bowel movements should pass without straining. Signs of constipation include going fewer than three times a week, hard or lumpy stools, straining, a feeling of **incomplete emptying**, or needing to press around the vagina or rectum to pass stool (“splinting”).

What Causes It

- **Low fiber and fluids**, and not enough activity
- **Ignoring the urge** to go
- **Medicines** — opioids, iron, some others
- **Pelvic-floor problems** — the muscles don’t relax to let stool out (“outlet” constipation)
- Slow movement of the colon

“Outlet” constipation is important: it is treated with **pelvic-floor therapy**, not just laxatives.

LEARN THE TERMS

Constipation

Infrequent, hard, or difficult-to-pass stools.

Fiber

Plant material that softens and bulks the stool.

Osmotic laxative

A gentle agent (like PEG) that draws water into the stool.

Outlet constipation

Trouble passing stool because the pelvic muscles don’t relax.

Biofeedback

Therapy that retrains the muscles to relax for a bowel movement.

Splinting

Pressing near the vagina or rectum to help stool pass.

WHY DOES MY UROLOGIST CARE? Because straining to pass hard stool puts pressure on the same support that holds up the bladder and pelvic organs — so constipation worsens prolapse and incontinence, and a full rectum can crowd the bladder. Fixing constipation often improves bladder symptoms too.

How It's Treated (Step by Step)

- 1 **Fiber, fluids, and activity** — build fiber up gradually (food or a supplement) with plenty of water.
- 2 **Toileting habits** — go when you feel the urge, use a **footstool** to raise the knees (squat position), and **don't strain**.
- 3 **Gentle laxatives** — an osmotic agent (like PEG) is safe for regular use if needed.
- 4 **Pelvic-floor therapy / biofeedback** for outlet-type constipation; and review any constipating medicines with your team.

A Healthy Routine

- Try to go at the **same time daily**, often after a meal.
- Give yourself unhurried time; never force or strain.
- Stay active — even a daily walk helps the bowel move.

Good to Know

- Add fiber **slowly** to avoid bloating, and drink enough water with it.
- Stimulant laxatives are fine occasionally; ask before using them daily.
- If straining is the main problem, the issue may be muscle coordination — therapy helps more than more laxatives.

Call your team if you have:

- **Blood in the stool**, or unexplained weight loss
- New constipation that won't improve, especially if you are older
- Severe belly pain, vomiting, or **no gas or stool** (possible blockage)

THREE THINGS TO REMEMBER

1. Constipation is common and matters for pelvic health — straining worsens prolapse and bladder leakage.
2. Fix it with fiber, fluids, activity, good toileting habits (footstool, no straining), and gentle laxatives as needed.
3. If straining persists, ask about pelvic-floor therapy. Report blood in the stool, weight loss, or a sudden severe change.